Treatment scope · NCCP 00382 This guide covers Lonsurf on its own for previously treated metastatic colorectal cancer under NCCP 00382a. It does not cover gastric cancer or Lonsurf with bevacizumab. Confirm the exact regimen on your prescription.

NCCP 00382 · VERSION 4

Trifluridine/tipiracil (Lonsurf) monotherapy

Take the prescribed morning and evening tablets on Days 1–5, have no Lonsurf on Days 6–7, then take morning and evening tablets on Days 8–12. Days 13–28 are tablet-free. A new 28-day cycle starts only when the team authorises it.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Low blood counts: infection, anaemia and bleeding

☐ Nausea, vomiting, diarrhoea and dehydration

☐ Kidney impairment can increase treatment toxicity

☐ Serious infection or febrile neutropenia

☐ Fatigue, reduced appetite and mouth soreness

☐ Pregnancy harm and important contraception requirements

Discuss the disease-control aim, expected benefit and uncertainty, alternatives including supportive care without this treatment, infection/bleeding risk, bowel symptoms and dehydration, oral dosing responsibilities, kidney/liver suitability, fertility, pregnancy prevention and your right to reconsider.

Individual benefit, alternatives and concerns

Agreed checks and next review

FBC, renal/liver profile and proteinuria dipstick. Confirm oral prescription, BSA/tablet band, previous treatment, symptoms and medicines. FBC, renal/liver profile and proteinuria dipstick before each cycle or as indicated. Review previous nadir, delay duration, current dose, GI exceptions, infection and oral adherence. Disease assessment as planned by Consultant; no fixed scan schedule.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.

SACTCheck · Developed by Dr Paul O'Brien · v0.84.24 · Sources checked 2026-10-29
Source reconciled; independent clinical, pharmacy and patient/PPI review pending